

STEP 2 OF 3 · THE BLUEPRINT · FOR APPROVAL

1,000 MEDICAL PROMPTS.

How I'm going to write your 1,000-prompt library. It combines the prompting guidelines of OpenAI, Anthropic, Google and Microsoft with the house style of the 12 Parabox prompt packs already in your Drive.

STEP 1 · DONE

Research the lab guidelines and build the SKILL.md

STEP 2 · THIS PDF

Study your Drive packs, merge both, lock the format

STEP 3 · NEXT

Write and QA the 1,000 prompts, part by part

12 of your packs, **read end to end.**

I searched your Drive for every prompt PDF you've published and read the medical and study ones in full. Here's what each one contributed to the blueprint.

YOUR PDF	WHAT I'M TAKING FROM IT
The Complete Thesis Prompts Topic to Manuscript · 6 prompts	The flagship format: USE IT / WHY / COPY THE PROMPT / RULES / HOW TO GET MORE OUT OF IT. Senior persona with a personality trait. MY CONTEXT fill-in block. [CITE] / [VERIFY] tags. The "hostile reviewer" and "AVOID THESE" endings.
The Research Prompt Pack 6 prompts incl. protocol	The protocol master prompt (ICMR, CTRI, IEC, consent form, Gantt timeline). The five non-negotiables page. "Run each stage in a fresh chat."
AI Research Prompts (3-prompt)	The phased prompt (Phase 1 → 2 → 3) inside one prompt, a tool-per-phase table, and an ethics-of-AI box.
AI in PG Thesis Parts 1–3 Topic Finder · Writing Guide · Methodology	Tool chains (Perplexity → Consensus → Semantic Scholar → Claude). Follow-up prompts. The "draft first, AI refines" golden rule. OpenEpi verification for sample size.
Thesis Stats Toolkit	Short in-tool prompts for Sheets, Julius AI and ChatGPT. "Check the assumption, name the test, report the exact p."
The Prompt Cheatcode	Your own P-R-O-M-P-T framework (Persona · Request · Output · Material · Parameters · Tweak), plus the Bad → Good → Better demo.
Ophthalmology Claude Prompts	The five exam archetypes: Topic Master, Viva Simulator, Comparison Table, PYQ Answer Writer, Flashcards. They work for any specialty.
Parabox Study Guide (NEET-PG / INI-CET)	NotebookLM audio, Custom GPT "answer only from my files" and a rapid-fire viva drill.
3 ChatGPT Prompts to Learn Any Topic	The tutor that checks understanding before moving on, the study-coach plan, and the whiteboard-logic exam mentor.
Medical Presentation Builder	The mixed-audience seminar deck: foundation recap, evidence corner, controversies, speaker notes and Vancouver references.
15 AI Cheat Codes	Format-driven prompts (flowchart, layers, decision matrix, roadmap, tree...) with a text fallback and a TRY IT WITH example line.
7 Prompts to Humanize AI · Consensus Workflow · Comic Flashcards · Handwritten Notes	Style-editing prompts, grounded search with verifiable citations, and visual study formats (for image tools).

NOT USED

Reel kits, image-conversion prompts, course and tool round-ups, and other people's PDFs shared with you. These aren't medical prompt packs, and some of them aren't your work.

The Parabox prompt DNA: 8 traits I'll keep in all 1,000.

Across every pack, your prompts share a signature. Your own line puts it best: "written the way a research supervisor thinks, not the way a chatbot answers."

01 A senior persona with credentials and a personality.

"A biostatistician with two decades of experience... openly critical of common misuse." "A strict but fair examiner." It's never just "You are an expert."

02 A MY CONTEXT block of labelled fill-in fields.

Specialty, programme and year, institution type, patient volume, resources you do and don't have. "The AI does not know your patients."

03 A numbered TASK that names exactly what comes out.

"Generate 8 candidate research questions... for each, give me exactly this: 1... 10."

04 A RULES block against fabrication, with named tags.

[CITE], [VERIFY], [ASSUMPTION — VERIFY FROM PUBLISHED DATA], [CITATION NEEDED — FIND SOURCE], [UNVERIFIED BACKGROUND].

05 India-first realism.

Government medical college, ICMR 2017, NFHS, CTRI, IEC, NEET-PG / INI-CET, 2-year MD timelines, limited lab infrastructure.

06 A self-critique ending.

"The three sentences a hostile reviewer would attack first", "AVOID THESE", "act as the ethics committee reviewer", "performance summary".

07 Wrapped in USE IT · WHY · HOW TO GET MORE OUT OF IT.

You tell the doctor when to use each prompt, why it exists, and how to push the output further.

08 An honest, direct voice.

"The AI drafts. You decide." "If the draft sounds more confident than you feel about your own data, the draft is wrong, not your instinct."

Design system (from your Thesis Prompts PDF)

COLOUR

Near-black #0A0808
background · Parabox orange
#FF641E · cream text #FBF4EC

TYPE

Poppins for headings and body
· *Lora italic* for intros and the
WHY line

ELEMENTS

Spaced-caps orange kickers,
big orange numbers, orange
left rule on prompt boxes,
orange cover band

This blueprint is set in that same system, so it doubles as a preview of how the final book will look.

Lab guidelines × your style: **what changes.**

Your packs already follow most of what OpenAI, Anthropic, Google and Microsoft recommend. Where they differ, here's what I'll do across all 1,000 prompts.

LAB GUIDELINE	YOUR PACKS TODAY	WHAT I'LL DO IN THE 1,000
Role / persona all four labs	✓ Already stronger than the labs' examples: seniority plus a personality trait.	Keep as is. Every prompt opens with a credentialed persona.
Give context & explain why Anthropic, Google, Microsoft	✓ The MY CONTEXT block. The WHY is written for the reader, not the model.	Keep the block, and add a short reason to key rules ("...because examiners check every number") so the model generalises.
Say what to do, not only what to avoid all four labs	Partly. Rules lean on "Do not...", though most already give the fix ([CITE]).	Every "Do not" gets paired with the action to take instead. The critical bans stay.
Long pasted material first, question last Anthropic: up to 30% better	Mixed. Some paste slots come after the task.	The [PASTE ...] slot always sits above the TASK block.
Name the output format all four labs	✓ Word counts, tables, "exactly this".	Keep. Every prompt names its shape, length and tone.
Examples (few-shot) OpenAI, Anthropic	Only in some packs (the variable list in Stats, TRY IT WITH in Cheat Codes).	Add a TRY IT WITH line to every Quick and Standard prompt, and a worked example to each Master prompt where it helps.
No contradictory instructions OpenAI GPT-5 guide	✓ None found.	An automated check during QA.
One job per prompt; chain big work Anthropic, OpenAI	Master prompts do 10–13 jobs, with a tip to run each stage in a fresh chat.	Keep a few Master prompts for full workflows. Most of the 1,000 do one job, with → NEXT pointers that chain them.
Iterate / follow-up turns all four labs	✓ The HOW TO GET MORE tips and the Tweak layer.	Keep, as a single PRO TIP line per prompt to save space.
Ideal length Google ~21 words for simple asks; longer for complex	Flagship prompts run 300–600 words. Ophthalmology / ChatGPT ones run 60–120.	Three tiers (next page). The Step 1 "Deep 150–300" tier is replaced by a longer Master tier to match your flagship prompts.

Built on your own **P-R-O-M-P-T**.

Your Prompt Cheatcode already covers every element the four labs require. Rather than bring in a new acronym, the 1,000 prompts are built on your framework, and every block maps to the lab guidance.

P Persona A senior expert with credentials and a trait Google Persona · OpenAI Identity · Claude role	R Request One clear task, as numbered steps Google Task · Microsoft Goal · OpenAI Instructions	O Output Shape, length, tone, tables, slides Google Format · Microsoft Expectations	M Material MY CONTEXT block + [PASTE] slot (first) Google Context · Microsoft Context + Source	P Parameters Rules, guidelines, [CITE] / [VERIFY] guards OpenAI constraints · Claude "say if unsure"	T Tweak Self-critique ending + PRO TIP + → NEXT All four labs: iterate
--	---	--	---	---	---

In each prompt, the blocks appear in the order the labs recommend (Persona → Material → Request → Parameters → Output → Tweak), with your labels: **MY CONTEXT · TASK · RULES · OUTPUT**.

Three tiers, **matched to the job**.

Quick 40–80 words One focused output: a differential, 20 flashcards, a mnemonic, one slide rewrite. Persona + Request + Output in 2–4 sentences, like your ChatGPT and Ophthalmology prompts. ≈ 300 prompts · 4 per page	Standard · default 100–200 words One real task done properly: a mini MY CONTEXT, a numbered TASK, one or two RULES, OUTPUT and a PRO TIP. Most of the library. ≈ 580 prompts · 2 per page	Master 250–450 words Your flagship format for full workflows: protocol, SAP, full ROL, full manuscript, full mock viva. The full USE IT / WHY / TIPS wrapper. ≈ 120 prompts · 1 per page
---	--	---

CONTEXT RULES (FROM STEP 1, NOW IN YOUR STYLE)

- 2–5 context fields the AI can't guess: level, specialty, setting, exam or journal, resources.
- Placeholders in **[SQUARE BRACKETS]** with a hint, like [e.g. Respiratory Medicine]. Five or fewer in Quick prompts; more are fine in the Master MY CONTEXT block.
- Anything pasted goes above the TASK. No contradictions. Say what to do, and name the output.

Sample cards: Quick & Standard.

These are real prompts that will go in the book, laid out as they'll appear.

921 Prioritised differential with can't-miss diagnoses

QUICK

PART 3 › 3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · STUDENT / RESIDENT

Act as a senior [SPECIALTY] consultant teaching on ward rounds. For this de-identified case, [PASTE AGE, SEX, COMPLAINT, KEY FINDINGS], list the 5 most likely diagnoses and the 2 can't-miss diagnoses. For each, give the one finding for and the one against, then the single next investigation that best separates the top two. Reason step by step. Say if the case lacks data to decide.

TRY IT WITH 58M, sudden breathlessness, pleuritic pain, HR 118, SpO2 89%, recent knee surgery.

512 Journal club: critical appraisal & presentation

STANDARD

PART 2 › 2.2 JOURNAL CLUB · RESIDENT

USE IT The week before you present, after you've chosen the paper.

PERSONA

You are a clinical epidemiologist who moderates a departmental journal club and is known for spotting the flaw everyone else missed.

MY CONTEXT

I am a [PG year] resident in [SPECIALTY], presenting to residents and faculty in [N] minutes.
[PASTE FULL TEXT OR ABSTRACT HERE]

TASK

1. Summarise the PICO and the study design in 4 lines. 2. Appraise it with the [CASP / JBI] checklist for this design, item by item. 3. Give 3 real strengths and the 3 limitations that most threaten the conclusion. 4. State the clinical bottom line for an Indian setting. 5. Give 5 discussion questions for the audience.

RULES

Use only the paper I pasted. If a detail isn't reported, write [NOT REPORTED] rather than assuming it, because the audience will check.

OUTPUT

A 12–15 slide outline: slide title, 3–5 bullets, one-line speaker note. End with the 3 questions faculty are most likely to ask me, with short answers.

PRO TIP Run it a second time with "act as the harshest faculty member". Those answers become your backup slides. →

NEXT #548 Turn this outline into slides.

Sample card: Master.

361 From interesting case to CARE-compliant case report

MASTER

PART 1 › 1.12 CASE REPORTS & CASE SERIES · RESIDENT / FACULTY

USE IT As soon as you suspect a case is unusual, while the notes, images and consent are still easy to get.

WHY Most case reports are rejected because they aren't novel or the timeline is unclear, not because the writing is weak. This prompt checks publishability first.

PERSONA

You are the case-reports editor of an indexed [SPECIALTY] journal. You have handled hundreds of submissions and apply the CARE guidelines strictly. You are blunt about whether a case is actually publishable.

MY CONTEXT

Specialty and my level: [e.g. MD Respiratory Medicine, PG year 2]

Setting: [tertiary / district / private]

De-identified case summary (age band, sex, presentation, key investigations with dates as Day 0, Day 3..., treatment, outcome, follow-up): [PASTE]

Why I think it is unusual: [state]

Target journal and its case-report word limit: [name / limit]

Patient consent for publication: [obtained / pending]

TASK

1. Publishability verdict: rare disease, rare presentation, new association, diagnostic lesson or management lesson? Score novelty out of 5 and justify it against my case.
2. Five title options that each contain the words "case report".
3. A CARE-structured draft: abstract (≤ 150 words), introduction, patient information, clinical findings, timeline table, diagnostic assessment (including the differential you excluded and how), therapeutic intervention, follow-up and outcome, discussion, patient perspective placeholder and learning points.
4. A CARE checklist table showing where each item appears and flagging anything missing.
5. The consent and de-identification checklist I need before submission.

RULES

Use only the clinical details I gave. Where something is missing, write [ASK ME: ...] instead of filling it in. Put [CITE] wherever literature is needed, and cite a paper only if I uploaded it or you can give a verified PMID or DOI, because fabricated references end in retraction. Keep every date relative (Day 0...) and remove anything identifying.

OUTPUT

Formal case-report register within my journal's word limit, then the two checklist tables.

TWEAK

Finish with the 3 comments a reviewer is most likely to make, and how to pre-empt each.

HOW TO GET MORE OUT OF IT

- › If the verdict says "not novel", reframe the case as a teaching image or a letter instead of forcing a full report.
- › Build the timeline table first. Reviewers read it before the text.
- › Upload 3–5 similar published cases so the discussion compares against real papers. → **NEXT** #366 Case report to conference poster.

Part 1 · Thesis, Research & Publication 400

#	SECTION	N	SAMPLE PROMPTS
1.1	Topic selection & research question	35	8 feasible topics from my context · FINER stress-test of my question · Clinical hunch → PICOT
1.2	Literature search strategy	30	PubMed MeSH search string · Broad vs narrow versions · Grey literature & CTRI checklist
1.3	Writing the Introduction	30	Four-movement introduction · Refine my draft (language only) · Burden-data sourcing checklist
1.4	Review of Literature	40	Thematic ROL skeleton · Extraction table from uploaded PDFs · Three-sentence gap statement
1.5	Aims, objectives & hypotheses	20	SMART objectives · Null and alternate hypotheses · Objective ↔ outcome alignment check
1.6	Study design & methodology	40	Choose the right design · Operational definitions for every variable · Criteria a second reviewer can apply
1.7	Sample size & statistics	45	Pre-specified SAP · "Which test?" decision table · Sample size with every assumption shown
1.8	Results, tables & figures	25	Table 1 layout · Results without interpretation · Best figure for my primary outcome
1.9	Discussion, conclusion & limitations	30	10-paragraph discussion · Limitations ranked by threat · Hostile-reviewer rewrite
1.10	Protocol, ethics & synopsis	20	IEC-ready ethics section (ICMR 2017) · Plain-language consent form · CTRI registration walk-through
1.11	Thesis → manuscript & publication	35	Thesis-to-paper cut plan · Journal fit check · Cover letter under 300 words · Response to reviewers
1.12	Case reports & case series	30	Is my case publishable? · CARE-compliant draft · Case series pattern and table builder
1.13	Conference abstracts, posters & oral papers	20	Abstract to the exact word limit · Thesis → 10-minute oral paper · Poster from my manuscript

WHERE YOUR EXISTING PROMPTS FIT

Your 6 flagship thesis prompts and the protocol prompt become the **Master** anchors for sections 1.1, 1.3, 1.4, 1.7, 1.9, 1.10 and 1.11. They'll be refreshed to the merged rules above (paired "Do not"s, paste slot first, TRY IT WITH line). The Standard and Quick prompts around each one break its work into single steps.

Part 2 · Seminars, Journal Clubs & Presentations 300

#	SECTION	N	SAMPLE PROMPTS
2.1	Seminar preparation	60	Seminar outline for any topic · Mixed-audience depth layering · Evidence corner: landmark trials
2.2	Journal club	60	Paper selection shortlist · CASP / JBI appraisal · Faculty questions I'll be asked
2.3	Slide design & structure	40	One idea per slide rewrite · Text → table or flowchart · Speaker notes for every slide
2.4	Conference oral presentations	35	8-minute timing plan · Opening hook · Rehearsal Q&A drill
2.5	Conference posters	35	Cut content to 800 words · Visual hierarchy brief for Canva · E-poster version
2.6	Case presentations & grand rounds	40	Grand-rounds case narrative · Case → teaching points · Questions the HOD will ask
2.7	Delivery, Q&A & teaching	30	Handling a question I can't answer · Teaching session for interns · Script feedback

Part 3 · Study, Exam & Viva Prep 300

#	SECTION	N	SAMPLE PROMPTS
3.1	Learning any topic	50	Teach me in four layers · Structured topic master · Tutor that checks my understanding
3.2	Study planning & revision	30	Day-by-day plan weighted to weak areas · Last-30-days plan · Recover a broken schedule
3.3	MCQ practice & exam strategy	50	20 exam-pattern MCQs · "Why was my answer wrong?" · PYQ trap analysis (NEET-PG, INI-CET, USMLE, PLAB, MRCP)
3.4	Long / short answer writing	30	Model long answer + examiner key points · 250-word short note · Diagram description
3.5	Viva & OSCE	50	Mock viva, one question at a time · OSCE station simulation · Spotter and instrument viva
3.6	Clinical reasoning & differentials	60	Prioritised DDX with can't-miss diagnoses · Next best investigation · Illness-script builder
3.7	Memory aids & high-yield	30	20 Anki-ready flashcards · Mnemonics · One-page high-yield sheet · Comparison table

How the final PDF is built.

Front matter

- › Cover in the Parabox style
- › **Read this first:** your 5 non-negotiables
- › **How to use each prompt:** brackets, fresh chat per stage, push back
- › **The P-R-O-M-P-T framework** with the Bad → Good → Better demo
- › **Quick-find index:** "I need to..." → prompt number

Body

- › 3 part openers with section lists
- › 27 section openers, each with a one-line WHY
- › 1,000 numbered prompt cards, with tier badges and → NEXT chains

Back matter

- › Tool-per-task table (Perplexity, Consensus, NotebookLM, Claude, ChatGPT, Julius...)
- › Ethics of AI in research box · closing note · disclaimer

Every prompt passes this QA check

- › Has all six P-R-O-M-P-T blocks (Quick prompts may merge them)
- › The persona has seniority and a trait
- › Word count is inside its tier
- › Paste slot sits above the TASK
- › Output shape and length are named
- › Every "Do not" comes with the action to take instead
- › Has an accuracy guard where facts matter: [CITE], [VERIFY], PMID/DOI rule
- › No patient identifiers, and the de-identification line is present on case prompts
- › No contradictions, and no near-duplicate of another prompt
- › Works unchanged in ChatGPT, Claude, Gemini and Copilot

SIZE ESTIMATE

≈ 300 Quick (4/page) + 580 Standard (2/page) + 120 Master (1/page) ≈ **480–520 pages** with openers. See decision 01.

Production plan

BATCH	WHAT YOU GET
Batch 1	Part 1 · Thesis, Research & Publication (400 prompts) as a PDF for review
Batch 2	Part 2 · Seminars, Journal Clubs & Presentations (300)
Batch 3	Part 3 · Study, Exam & Viva Prep (300), then the combined 1,000-prompt book with index

Each batch goes through automated checks (word counts, bracket counts, duplicates, required blocks), then a read-through before you see it.

Four decisions before I start.

Each one has a recommendation. Reply "go with the recommendations" or change any of them.

01 One book or three volumes?

At about 500 pages, a single PDF is heavy to share on WhatsApp or through DMs. Three volumes (one per part) plus a combined edition are easier to share and let you release one at a time.

Recommended: three volumes plus a combined edition.

02 Reuse your existing prompts?

About 40 of your published prompts (thesis, protocol, ophthalmology exam set, study guide) could become refreshed anchors inside the 1,000, or the library could be 100% new.

Recommended: reuse and refresh them as Master anchors, clearly the upgraded versions (about 4% of the library).

03 Audience scope and exam list

Your packs are India-first (ICMR, CTRI, NEET-PG / INI-CET). Should prompts also carry international variants such as USMLE, PLAB, MRCP and FCPS?

Recommended: India-first defaults, with international exams offered as options inside the brackets, e.g. [NEET-PG / INI-CET / USMLE / PLAB / MRCP].

04 Tier mix and split

300 Quick / 580 Standard / 120 Master, split 400 Thesis / 300 Seminars / 300 Study.

Recommended: keep as proposed.

Sources

Your Drive (12 PDFs)	The Complete Thesis Prompts · Research Prompt Pack · AI Research Prompts · AI PG Thesis Parts 1–3 · Thesis Stats Toolkit · Prompt Cheatcode · Ophthalmology Claude Prompts · Parabox Study Guide · 3 ChatGPT Prompts · Medical Presentation Builder · 15 AI Cheat Codes · 7 Humanize Prompts · Consensus Workflow
Anthropic	platform.claude.com/docs/en/build-with-claude/prompt-engineering/claude-prompting-best-practices
OpenAI	GPT-5 prompting guide (OpenAI Cookbook) · developers.openai.com/api/docs/guides/prompt-engineering
Google	ai.google.dev/gemini-api/docs/prompting-strategies · Gemini for Workspace prompt tips
Microsoft	Microsoft 365 Copilot: writing prompts · Azure OpenAI / Foundry prompt engineering techniques
Medical	JMIR 2025, Prompt Engineering in Clinical Practice: Tutorial for Clinicians